

The politics of peer review and preprints in the real world

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This week has been one in which my personal and professional lives have aligned in interesting ways. As you may know, one strand of my work focuses on the study of academic publishing, including [peer review](#). This has featured discussions of the idea of “[excellence](#)” but also how preprints are viewed in the world.

Another part of my life concerns a strand of disability activism around the [non-availability of the drug Evusheld in the UK](#). Bear with me if you’ve already heard me harp on about this. Evusheld is a combination of two long-acting antibodies (tixagevimab and cilgavimab). It’s a drug designed to protect clinically vulnerable people against Covid in cases where vaccines don’t work. A good example is people who have been on the chemotherapy drug Rituximab, which causes much worse Covid outcomes and also reduces vaccine efficacy. Another example is people with primary or secondary immunodeficiency. Evusheld is given prophylactically (that is: in advance) to stop people in this situation developing severe Covid in the first place. Evusheld was authorised by the UK’s MHRA on the 17th March 2022. This means that it met the UK regulatory standards of safety, quality and effectiveness. 32 other countries are using the drug. But the UK still refuses to do so. This drug would change my life and make it safe for me to end shielding. As it stands, I instead am approaching 1,000 days of isolation. The only reason I go anywhere in public is for crucial medical appointments.

Our patient campaigning group has written to the Secretary of State for Health in the UK several times on this matter. This week, though, they finally published [their response to the letters that I drafted](#). What's interesting here ("interesting") is the way that peer review, preprints, and limitations are used within a political context.

In the letter, where we had sent them several new studies, the Department of Health mounted the following criticisms:

- "it should also be noted that most of these studies are pre-prints (therefore have not been scrutinised through peer review)"
- They also highlighted the "limitations" section in each of the papers

One of our charity partners, Blood Cancer UK, has written a response to this letter, and it's worth quoting some of their excellent reply. On peer review and preprints, they write:

In addition, that many of the real-world study publications have yet to be peer-reviewed is also, we believe, not a legitimate reason to prematurely dismiss them. Albeit a rigorous and important mechanism in the publication process, it is widely accepted that peer-review is not a perfect mechanism for managing research integrity? and that it is possible to review and make judgment on evidence before it is peer-reviewed. The argument, therefore, that evidence published in pre-prints is fundamentally flawed because they have yet to undergo the peer-review process is one with which we disagree. Indeed, the Department itself used data which, at the time, was presented in pre-prints to inform their public-facing communications outlining the efficacy of the COVID-19 vaccines in the UK population. One of the largest pre-print servers for health sciences research - medRxiv - was established by Yale University, the British Medical Journal, and others specifically to "improve the openness and accessibility of scientific findings, enhance collaboration among researchers...through more timely reporting of completed research." While pre-prints are not intended to guide clinical practice, they played - and continue to play - a crucial role in the public health response to the pandemic where key information would have otherwise been slow to reach publication.

They also pick the DHSC up for citing the limitations in the positive studies that show Evusheld working... while ignoring the limitations in the studies they favour:

Citing limitations for those studies which demonstrate Evusheld's effectiveness, while not referencing the limitations of lab-based studies is misleading. It is a basic requirement of manuscript authors to appraise the strengths and weaknesses of their studies; the stated limitations do not reduce the significance of their research findings, but rather acknowledge the potential shortcomings of the study to ensure integrity and honesty. Indeed, the Oxford COVID Vaccine Trial Group publication' that helped lead to the rapid clinical introduction of the AstraZeneca Oxford Covid vaccine cited limitations including a short follow-up, small participant numbers in the prime-boost group, the single-blinded design, and the fact that the findings are not easily generalisable due to the relative youth and health of participants, as well as that they were majority white. Despite these methodological limitations, the vaccine was – quite rightly - rolled out at speed.

It's fair to say that the DHSC are not treading a very impressive line here in the way they appraise and marshall evidence. They're quite happy with preprints... sometimes. And they are quite happy with the limitations of studies... sometimes. But in this case, when they wish to discredit certain research, they suddenly reach for these aspects.

In other words, in the ongoing tussle between science and politics, which has underwritten the entirety of the pandemic response, features such as peer review play an important legitimizing discursive role, rightly or wrongly.